

ResearchStack Starter

A practical research launch system for Australian medical students who want to find projects, contact supervisors, and build CV-ready research outputs without guessing.

Founder draft

This compiled PDF includes the full working product source: guides, scripts, templates, tracker previews, sales notes, and content seeds.

[Supervisor outreach](#)[Audit / QI starter](#)[Literature search](#)[CV evidence](#)[Publication roadmap](#)[AI prompt vault](#)

Education and organisation support only. Students must check local university, hospital, supervisor, governance, and HREC requirements.

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ResearchStack Starter

ResearchStack Starter is a self-serve research launch system for Australian medical students who want to move from "I should do research" to a clear project pipeline, supervisor outreach plan, and first publishable output pathway.

This folder is the source pack. It can later be converted into PDFs, Notion pages, Google Sheets, Lemon Squeezy files, and website copy.

Product Promise

In 30 days, the student should know:

- what type of research project suits their current stage
- how to find and contact potential supervisors
- how to track opportunities without losing follow-ups
- how to run a literature search without pretending they are already an academic
- how to turn a project into CV-ready evidence
- what to ask before joining a project
- what "audit/QI vs research" usually means in an Australian hospital context
- what their next concrete research action is this week

Included Files

Start here:

- 01-product-positioning.md
- 02-30-day-research-sprint.md
- 03-project-type-playbook.md
- 04-australian-ethics-qi-audit-guide.md
- 05-literature-search-starter.md
- 06-output-and-publication-roadmap.md
- 07-ai-prompt-vault.md

Scripts:

- scripts/research-outreach-email-scripts.md
- scripts/supervisor-meeting-scripts.md
- scripts/follow-up-and-rescue-scripts.md

Templates:

- templates/case-report-template.md
- templates/audit-qi-project-template.md
- templates/abstract-template.md
- templates/manuscript-section-template.md

Trackers:

- trackers/research_pipeline.csv
- trackers/supervisor_crm.csv
- trackers/literature_matrix.csv
- trackers/conference_journal_tracker.csv
- trackers/authorship_contribution_tracker.csv
- trackers/data_extraction_template.csv

Important Note

This product is education and organisation support only. It is not legal, ethics, university, hospital, or medical advice. Students should check their local university, hospital research office, supervisor, and HREC requirements before collecting data, contacting patients, submitting abstracts, or publishing.

ResearchStack Starter: Product Positioning

One-Line Offer

The beginner research system for Australian medical students who want to find projects, contact supervisors, track progress, and turn research into CV-ready outputs without guessing.

Who It Is For

- Australian medical students in pre-clinical or clinical years
- Students who know research matters but do not know where to start
- Students aiming for competitive pathways where research, audits, presentations, publications, and leadership matter
- Students who have emailed one or two doctors before and heard nothing back
- Students who have joined a project but feel unclear about the next action

Who It Is Not For

- PhD-level researchers
- Students who already have multiple first-author papers and a strong supervisor network
- Anyone looking for guaranteed publication
- Anyone wanting shortcuts around ethics, authorship, or academic integrity

Core Outcome

By the end of the starter system, the student should have:

- a shortlist of realistic project types
- a list of 10-20 possible supervisors, teams, or project sources
- 3-5 tailored outreach emails ready to send
- a research pipeline tracker
- one project brief drafted
- one follow-up rhythm
- a CV evidence log for research outputs
- a clear understanding of what to ask before committing

Product Levels

ResearchStack Starter

Self-serve system. No video required at launch.

Includes:

- starter guide
- project type playbook
- outreach scripts
- literature search worksheet
- audit/QI guide
- case report and abstract templates
- trackers
- AI prompt vault

Target price:

- Launch: AUD 197-299
- Bundle with MedStack OS: AUD 199-249 for founding access
- Future with feedback/cohort: AUD 499-999

ResearchStack Pro

Future higher-ticket offer.

Could include:

- recorded walkthroughs
- live office hours
- project idea review
- CV/research profile review
- supervisor email feedback
- abstract/manuscript feedback
- templates for specific specialties

Why This Can Sell

The pain is specific:

- "I need research for my CV but do not know how to start."
- "I do not know who to email."
- "I do not know what counts as a good project."
- "I do not know how to follow up without annoying people."
- "I joined a project and now nothing is happening."
- "I have done work but do not know how to turn it into an abstract, poster, or CV point."

The product should not promise publication. It should promise structure, momentum, and fewer beginner mistakes.

Main Sales Angles

Angle 1: The Research Starting Problem

Most students are told research matters, but nobody gives them the operating system for finding, starting, and finishing projects.

Angle 2: Supervisor Outreach

Students lose months because they send vague emails or wait for opportunities instead of building a pipeline.

Angle 3: CV Evidence

Research only helps if it becomes traceable evidence: role, contribution, abstract, poster, presentation, manuscript, publication, referee, or lesson.

Angle 4: Australia-Specific Context

Australian students need to understand audits, quality improvement, HREC, hospital governance, specialty CV expectations, and local conference opportunities.

30-Day Research Sprint

This sprint is designed for a medical student starting from zero or near-zero research momentum.

The goal is not to publish in 30 days. The goal is to build a real research pipeline, contact the right people, and begin one realistic project.

Week 1: Choose Your Lane

Day 1: Define Your Research Direction

Pick 1-2 broad areas:

- surgery
- medicine
- emergency
- paediatrics
- psychiatry
- public health
- medical education
- rural health
- digital health / AI
- women's health
- oncology
- critical care

Write:

- I am interested in:
- I would be happy to work on:
- I do not want to work on:
- My current experience level:
- My realistic weekly time:

Day 2: Choose Project Types

Rank these by realism:

- case report
- audit or quality improvement project
- retrospective chart review
- literature review
- systematic review
- survey study
- medical education project
- database project
- conference abstract
- poster presentation

Beginner recommendation:

- case report if you have a good clinical case and a supportive doctor
- audit/QI if your hospital team has a clear clinical process gap
- literature review if you can access a supervisor and narrow question
- retrospective project only if data access and approvals are realistic

Day 3: Build Your Supervisor List

Find 10-20 people across:

- university academics
- hospital consultants

- registrars involved in research
- specialty society contacts
- student research societies
- supervisors from previous placements
- authors of local papers in your interest area

Log them in `trackers/supervisor_crm.csv`.

Day 4: Read Before You Email

For each top contact, find:

- their specialty
- 1-2 recent papers or projects
- hospital/university affiliation
- why their work is relevant to you
- one specific sentence you can mention in the email

Day 5: Write Your First 5 Emails

Use `scripts/research-outreach-email-scripts.md`.

Do not send the same generic email to everyone. Tailor the first 2-3 lines.

Day 6: Send and Track

Send 5 emails.

Track:

- date sent
- contact
- topic
- response
- follow-up date
- next action

Day 7: Review

Ask:

- Did I send enough?
- Were they specific?
- Did I offer realistic help?
- Did I make it easy for the doctor to reply?

Week 2: Create Project Options

Day 8-10: Find 3 Project Ideas

Use:

- recent journal articles
- local hospital quality gaps
- guideline updates
- common clinical problems
- unanswered questions from placement
- conference abstract books
- supervisor publications

For each idea:

- question
- project type
- possible data source
- possible output

- likely approval needed
- who could supervise it

Day 11-12: Prepare a Meeting Brief

For each meeting, create:

- 3-sentence intro
- why you are interested
- your weekly availability
- project types you can help with
- one example task you could do
- questions for the supervisor

Use `scripts/supervisor-meeting-scripts.md`.

Day 13-14: Follow Up

Follow up with anyone who has not replied after 7-10 days.

Use `scripts/follow-up-and-rescue-scripts.md`.

Week 3: Commit to One Real Project

Day 15-17: Score Opportunities

Score each possible project:

- clear supervisor
- clear question
- clear next action
- realistic timeline
- realistic approvals
- student role is meaningful
- output pathway exists

Avoid projects where:

- nobody knows the question
- there is no data access
- you are promised authorship vaguely
- the timeline is indefinite
- you cannot identify your role

Day 18-21: Set Up the Project

Create:

- project brief
- timeline
- authorship/contribution tracker
- literature matrix
- next 3 actions
- meeting rhythm

Use:

- `trackers/research_pipeline.csv`
- `trackers/authorship_contribution_tracker.csv`
- `trackers/literature_matrix.csv`

Week 4: Produce Evidence

Day 22-24: Start the Literature Search

Define:

- population
- problem
- intervention/exposure
- comparator if relevant
- outcome
- study type

Build 2-3 search strings.

Day 25-27: Create First Output

Pick one:

- one-page project brief
- literature summary table
- case report draft
- audit criteria table
- abstract skeleton
- data extraction sheet

Day 28-30: Lock Your Research System

Update:

- project tracker
- supervisor CRM
- next action
- CV evidence log
- follow-up dates

End with:

- one active project or one strong lead
- one next meeting or follow-up
- one draft output
- one CV evidence entry

Project Type Playbook

Not every research project is equal for a medical student. The best project is the one that has a clear question, realistic approvals, available supervision, and an output pathway.

Fast Comparison

Project type	Beginner fit	Typical output	Main risk
Case report	High	Abstract, poster, short paper	Needs a genuinely reportable case and consent/local process
Audit/QI	High	Poster, presentation, local change, paper sometimes	Needs governance and clear standards
Literature review	Medium	Review, presentation, background section	Can become too broad
Systematic review	Medium	Abstract, manuscript	Time-consuming, methods-heavy
Retrospective chart review	Medium	Abstract, poster, paper	Data access, ethics/governance
Survey study	Medium	Abstract, paper	Poor response rate, survey design
Prospective clinical study	Low	Paper	Often too slow for beginners
Lab/basic science	Variable	Paper, poster	Training and time required
Medical education project	High	Abstract, presentation, paper	Needs clear intervention/evaluation

Case Report

Best when:

- there is an unusual presentation, complication, diagnostic lesson, treatment issue, or systems lesson
- a clinician involved in the case is willing to supervise
- consent and local publication requirements can be handled properly

Student tasks:

- draft timeline
- collect de-identified clinical details under supervision
- literature search
- draft background/discussion
- prepare figures/table
- format to journal guidelines

Before starting, ask:

- Has consent been obtained or what is the local process?
- Who will be corresponding author?
- What journal or conference are we aiming for?
- What is my expected contribution?

Audit / Quality Improvement

Best when:

- there is a guideline, standard, or policy to compare practice against
- the data source is realistic
- the project can lead to local improvement

Student tasks:

- define criteria
- build data collection tool
- extract data if approved
- summarise baseline performance

- help design intervention
- prepare poster/presentation

Before starting, ask:

- Is this audit/QI or research in this hospital's process?
- Does it need HREC review, governance registration, or quality approval?
- Who owns the data?
- What is the output plan?

Literature Review

Best when:

- the question is narrow
- the supervisor knows the field
- there is a clear output plan

Student tasks:

- search strategy
- screening articles
- summary table
- background writing
- reference management

Common mistake:

- picking a topic like "AI in surgery" instead of a focused question.

Better:

- "Use of AI-assisted triage tools for emergency department waiting time prediction: a scoping review."

Systematic Review

Best when:

- the question is answerable
- the team has methods support
- the student has time

Before starting, ask:

- Has this review already been done?
- Is there a protocol?
- What database access do we have?
- How many reviewers are involved?
- Will we use Covidence, Rayyan, Excel, or another tool?
- What reporting guideline applies?

Retrospective Chart Review

Best when:

- the clinical team already knows the data exists
- the dataset is accessible
- approvals are realistic
- the question is clear

Student tasks:

- data dictionary
- data extraction sheet
- literature background
- cleaning data under supervision

- abstract/manuscript drafting

Main risk:

- getting stuck waiting for approvals or data access.

Medical Education Project

Best when:

- you can evaluate teaching, simulation, curriculum, feedback, or student experience
- the intervention is realistic
- ethics/governance is considered early

Possible outputs:

- conference abstract
- poster
- education journal paper
- student society presentation

How To Pick Your First Project

Choose the project with the best combination of:

- clear supervisor
- clear next action
- short feedback loop
- defined output
- realistic approvals
- useful CV evidence

For most students, the best first project is usually:

1. case report
2. audit/QI
3. focused literature review
4. medical education project

Australian Ethics, QI, Audit, and Governance Guide

This is a beginner-friendly orientation only. It is not ethics, legal, hospital, or university advice.

Australian medical students should always check the relevant hospital research office, university, HREC, supervisor, and local policies before collecting data, accessing patient records, contacting patients, or submitting work publicly.

The Core Distinction

Many student projects sit somewhere between:

- research
- quality assurance
- quality improvement
- clinical audit
- evaluation
- case report

The label matters because it can change:

- what approvals are needed
- who can access data
- whether patients need consent
- whether the project can be published
- which governance office needs to review it

Research

Usually aims to generate new knowledge that can apply beyond the local setting.

Examples:

- retrospective study comparing outcomes
- survey study designed for publication
- systematic review
- prospective study
- intervention study

Possible requirements:

- HREC review or exemption pathway
- site governance
- data access approval
- supervisor oversight
- consent or waiver depending on design

Audit

Usually compares current practice against an existing standard, guideline, or benchmark.

Examples:

- Are VTE risk assessments completed according to hospital policy?
- Are antibiotics given within the recommended time window?
- Are discharge summaries completed within local target time?

Possible requirements:

- audit registration
- quality assurance approval
- governance review
- permission to access records

- supervisor responsibility

Quality Improvement

Usually aims to improve a local system or process.

Examples:

- introducing a checklist
- improving documentation
- reducing delays
- improving handover quality

Possible requirements:

- QI registration
- governance review
- local department approval
- ethics review if publication/generalisation/data risk is involved

Case Reports

Case reports often require:

- supervisor involvement
- patient consent or local consent process
- de-identification
- journal/conference requirements
- hospital policy compliance

Never assume a case is publishable just because names are removed.

What To Ask Before Starting Any Project

Ask your supervisor:

- Is this research, audit, QI, evaluation, or a case report?
- What approval pathway applies at this hospital/university?
- Who is responsible for approval?
- Can I access the data, and under what conditions?
- Can this be submitted to a conference or journal?
- Do we need patient consent?
- Who will be listed as authors or contributors?
- What is the expected output?
- What is the target timeline?

Red Flags

Be careful if:

- someone asks you to access patient records without a clear approval process
- you are told "just grab the data" without governance
- authorship is vague
- there is no supervisor accountability
- the project uses identifiable patient information without clear permission
- the team wants publication but has not considered approvals

Student-Safe Principle

If unsure, pause and ask:

- supervisor

- hospital research office
- university research office
- HREC contact
- clinical governance / quality unit

It is better to clarify early than to spend months on a project that cannot be submitted.

Useful Official Starting Points

- NHMRC: Ethical considerations in quality assurance and evaluation activities
- NHMRC: National Statement on Ethical Conduct in Human Research
- EQUATOR Network: reporting guidelines
- CARE: case report guideline
- STROBE: observational study guideline
- PRISMA: systematic review guideline

Literature Search Starter

Most beginners do one of two things:

- search one vague phrase and save random papers
- collect too many papers without knowing what question they are answering

This guide keeps it simple.

Step 1: Define the Question

Use PICO when relevant:

- Population: who or what group?
- Intervention/exposure: what treatment, exposure, tool, process, or issue?
- Comparator: compared with what?
- Outcome: what result matters?

For reviews that do not fit PICO, use:

- Population
- Concept
- Context

Step 2: Turn the Question Into Search Blocks

Example topic:

"AI-assisted triage tools in emergency departments."

Search blocks:

- AI terms: artificial intelligence, machine learning, prediction model, algorithm
- triage terms: triage, risk stratification, prioritisation
- setting terms: emergency department, emergency medicine, ED

Step 3: Build Search Strings

Basic structure:

("artificial intelligence" OR "machine learning" OR algorithm) AND (triage OR "risk stratification") AND ("emergency department" OR "emergency medicine")

Step 4: Search More Than One Place

Beginner databases/tools:

- PubMed
- Google Scholar
- university library search
- Cochrane Library if relevant
- specialty journal websites
- reference lists from key papers

If doing a formal review, ask a librarian or supervisor before finalising the search.

Step 5: Track What You Search

Record:

- database
- date
- exact search string

- filters used
- number of results
- notes

Use `trackers/literature_matrix.csv`.

Step 6: Screen Quickly

First pass:

- title
- abstract
- relevance
- study type
- year
- population

Keep:

- highly relevant studies
- recent reviews
- guideline papers
- landmark papers
- papers from your supervisor/local team if relevant

Remove:

- off-topic papers
- wrong population
- wrong setting
- opinion pieces unless useful for background

Step 7: Extract Useful Details

For each important paper:

- citation
- study design
- population
- sample size
- intervention/exposure
- outcome
- key finding
- limitation
- how it helps your project

Step 8: Turn Reading Into Writing

Every paper should have a job.

Jobs:

- justifies the problem
- explains current evidence
- shows a gap
- supports your method
- compares your result
- strengthens discussion

If you cannot explain why a paper is in your table, remove it.

AI Use

AI can help summarise and structure your notes, but do not rely on it to verify citations or invent references.

Always check:

- title
- authors
- journal
- year
- DOI
- actual conclusions

Output and Publication Roadmap

Research helps your CV when it becomes clear evidence of contribution.

The aim is not only "publication." Outputs can build towards publication.

Output Ladder

Level 1: Project Evidence

- project brief
- literature matrix
- data extraction sheet
- audit criteria table
- ethics/governance submission support
- meeting notes
- contribution log

Level 2: Local Output

- department presentation
- hospital audit meeting
- student society presentation
- teaching session
- poster draft

Level 3: Conference Output

- abstract
- poster
- oral presentation
- conference proceedings
- prize nomination

Level 4: Publication Output

- case report
- audit/QI manuscript
- review article
- original research paper
- letter/commentary

How To Turn a Project Into CV Evidence

Track:

- project title
- supervisor
- institution
- your role
- skills used
- outputs created
- presentation/submission/publication status
- dates
- evidence link
- referee/contact

CV phrasing examples:

- Assisted with literature review and data extraction for a retrospective audit of [topic].
- Co-authored abstract submitted to [conference].
- Presented findings at [department meeting/conference].
- Developed data collection tool for quality improvement project assessing [standard].
- Drafted background and discussion sections for a case report on [topic].

What To Clarify Early

Before doing major work, ask:

- What output are we aiming for?
- Who is responsible for each section?
- What is the timeline?
- What are the authorship expectations?
- Where will this be submitted?
- What happens if the project stalls?

Authorship Basics

Authorship should reflect meaningful contribution and follow journal/institutional expectations.

Do not assume:

- "I collected data" always equals authorship
- "I know the supervisor" means authorship
- "I started the project" means first author

Use the contribution tracker so your work is visible.

Submission Targets

Possible targets:

- university research day
- hospital audit meeting
- specialty society conference
- student medical journal
- case report journal
- specialty conference
- local poster day

Pick the target early so the format does not surprise you later.

Beginner Rule

Every active project should have one next output.

Examples:

- case report: submit abstract by July
- audit: present baseline data at department meeting
- review: complete first screening pass
- retrospective study: finalise data dictionary

AI Prompt Vault

Use these prompts to move faster, not to fake expertise. Always verify facts, references, ethics requirements, and clinical claims with real sources and your supervisor.

Find a Focused Research Question

I am an Australian medical student interested in [specialty/topic]. I have [weekly time] available and [experience level]. Give me 10 realistic beginner research project ideas. For each idea, include project type, possible supervisor, likely data source, possible output, and biggest risk.

Narrow a Broad Topic

Turn this broad topic into 10 narrower research questions suitable for a medical student: [topic]. Prefer questions that could become a case report, audit/QI project, literature review, or retrospective study.

Supervisor Shortlist

I want to find supervisors in [specialty/location/university/hospital]. Create a search strategy for finding potential supervisors using PubMed, Google Scholar, hospital websites, university profiles, and specialty societies. Include what details I should record in a CRM.

Tailor a Research Outreach Email

Draft a concise email to Dr [Surname], a [specialty] consultant/researcher. I am a [year] medical student at [university]. I am interested in [topic] because [reason]. I found their work on [paper/project]. I want to ask whether there are any research projects I could assist with. Tone: respectful, specific, not too long, Australian medical student style.

Improve an Outreach Email

Improve this email so it sounds specific, professional, and easy to reply to. Keep it concise and do not exaggerate my experience:

[paste email]

Prepare for Supervisor Meeting

I have a meeting with a potential research supervisor about [topic]. Create a 15-minute meeting agenda, questions to ask, and a short script for introducing myself. Include questions about approvals, authorship, timeline, data access, and output plan.

Literature Search Terms

For this research question: [question], create search term blocks using synonyms and related terms. Then create three PubMed-style search strings: broad, balanced, and narrow.

Summarise Papers Into a Matrix

Create a literature matrix structure for papers on [topic]. Columns should include citation, study design, population, intervention/exposure, outcome, key finding, limitations, and how it informs my project.

Turn Notes Into an Abstract Draft

Using the notes below, draft a structured abstract with Background, Methods, Results, and Conclusion. Mark missing information clearly instead of inventing details.

[paste notes]

CV Evidence Wording

Turn this research involvement into 5 professional CV bullet points. Do not overstate my role. Include versions for contribution, presentation, abstract, and publication if applicable.

[paste details]

Project Rescue

This research project has stalled. Details: [project, supervisor, last contact, current blocker, my role]. Create a respectful follow-up email and a rescue plan with three possible next actions.

Research Outreach Email Scripts

Keep emails short, specific, and easy to reply to.

Cold Email: No Prior Relationship

Subject: Medical student research opportunity

Dear Dr [Surname],

My name is [Name], and I am a [year level] medical student at [University].

I am interested in [specialty/area], particularly [specific topic]. I came across your work on [paper/project/service/clinic area], and I would be keen to learn whether there may be any research, audit, quality improvement, or case report projects I could assist with.

I am happy to help with tasks such as literature searching, data collection, reference management, drafting sections, or preparing abstracts if useful.

If you are available, I would be grateful for the chance to introduce myself briefly by email, Zoom, or in person.

Kind regards,

[Name] [Phone/email]

Warm Email: Placement Doctor

Subject: Research / audit opportunity

Dear Dr [Surname],

I hope you are well. My name is [Name], and I was recently attached to [team/rotation] as a [year level] medical student.

I really enjoyed [specific part of the term], and I wanted to ask whether there are any research, audit, QI, or case report projects in the department that a student could assist with.

I would be happy to help with literature searches, data collection, drafting, or abstract preparation, depending on what would be most useful.

Thank you again for the teaching during the term.

Kind regards,

[Name]

Email After Seeing a Paper

Subject: Medical student research enquiry regarding [topic]

Dear Dr [Surname],

My name is [Name], and I am a [year level] medical student at [University].

I recently read your paper on [paper/topic], and I found [specific point] particularly interesting. I am interested in [specialty/area] and wanted to ask whether your team has any current or upcoming projects that a medical student could contribute to.

I am especially keen to develop skills in literature review, data extraction, audit/QI work, and academic writing.

If useful, I would be happy to send through my CV or briefly introduce myself.

Kind regards,

[Name]

Follow-Up After No Reply

Subject: Follow-up: medical student research opportunity

Dear Dr [Surname],

I hope you are well. I just wanted to follow up on my email below regarding possible research, audit, or QI opportunities.

I completely understand if now is not a suitable time. If there is someone else in the team who may be better to contact, I would be grateful for any direction.

Kind regards,

[Name]

Short Referral Email

Subject: Medical student research enquiry

Dear Dr [Surname],

My name is [Name], and I am a [year level] medical student at [University]. Dr [Referrer] suggested I contact you because of your work in [area].

I am interested in getting involved in research, audit, QI, or case report work, and I would be grateful for any opportunity to assist the team.

I am happy to help with literature searches, data collection, drafting, or abstract preparation.

Kind regards,

[Name]

What Makes the Email Work

- It names who you are.
- It shows a real reason for contacting them.
- It offers practical help.
- It does not demand a project.
- It makes replying easy.

Supervisor Meeting Scripts

15-Minute Meeting Agenda

1. Introduce yourself
2. Explain your interest and availability
3. Ask what projects are active or realistic
4. Clarify approval/governance requirements
5. Clarify your role
6. Clarify output and timeline
7. Agree next action

Introduction Script

Thank you for meeting with me. I am a [year level] medical student at [University], and I am interested in [specialty/topic]. I am hoping to get involved in a realistic research, audit, QI, or case report project where I can contribute consistently and learn the process properly.

At the moment, I can commit around [time] per week. I am comfortable helping with literature searches, data extraction, reference management, drafting, and abstract preparation.

Questions To Ask

Project:

- What projects are currently active in the team?
- Are there any projects suitable for a medical student?
- Is the question already defined?
- What is the expected timeline?

Approvals:

- Is this research, audit, QI, evaluation, or case report?
- What approvals or governance steps are required?
- Has anything already been submitted or approved?
- Can a student access the data?

Role:

- What would my first task be?
- How often should I update you?
- Who else is involved?
- What skills should I learn before starting?

Output:

- Are we aiming for a poster, abstract, presentation, or manuscript?
- Is there a target conference or journal?
- How will contribution/authorship be decided?

End-of-Meeting Close

Thank you, that sounds helpful. To make sure I understood correctly, my next step is to [next action] by [date]. I will update [person] once that is done, and we can then decide the next stage.

After-Meeting Email

Subject: Thank you and next steps

Dear Dr [Surname],

Thank you for meeting with me today.

My understanding is that the next step is for me to [task] by [date]. I will also [second task if relevant].

I will keep track of progress and send through an update once completed.

Kind regards,

[Name]

Follow-Up and Rescue Scripts

Follow-Up After First Email

Subject: Follow-up regarding research opportunity

Dear Dr [Surname],

I hope you are well. I just wanted to follow up on my previous email regarding possible research, audit, QI, or case report opportunities.

I completely understand if you are busy. If there is someone else in the team who may be more appropriate to contact, I would be grateful for any direction.

Kind regards,

[Name]

Follow-Up After a Meeting

Subject: Follow-up regarding [project/topic]

Dear Dr [Surname],

I hope you are well. I wanted to follow up on [project/topic] and ask whether there are any next steps I can help with.

I am happy to work on [literature search/data collection/draft abstract/table/manuscript section] if useful.

No rush at all, but I wanted to stay proactive and keep the project moving where I can.

Kind regards,

[Name]

Project Has Stalled

Subject: Checking next steps for [project/topic]

Dear Dr [Surname],

I hope you are well. I wanted to check in regarding [project/topic].

My current understanding is that the next step is [blocker/next step]. I am happy to help move this forward by [specific task], or if priorities have changed, I completely understand.

Please let me know what would be most useful.

Kind regards,

[Name]

Asking for Clear Direction

Subject: Next steps for [project/topic]

Dear Dr [Surname],

I hope you are well. I wanted to make sure I am using my time effectively on [project/topic].

Would it be helpful for me to focus on one of the following next steps?

1. update the literature search
2. prepare a summary table
3. draft an abstract outline
4. clean the data extraction sheet
5. pause until approvals/data access are confirmed

Happy to follow whichever direction is most useful.

Kind regards,

[Name]

Withdrawing Politely

Subject: [Project/topic]

Dear Dr [Surname],

Thank you again for the opportunity to be involved in [project/topic].

At this stage, I do not think I can contribute to the standard the project deserves alongside my current commitments, so I wanted to be upfront rather than risk delaying the team.

I am grateful for the opportunity and hope to work together in the future if timing is better.

Kind regards,

[Name]

Case Report Template

Working Title

[Condition/presentation/intervention]: a case report

Why This Case Matters

- What is unusual?
- What is the learning point?
- Why would a reader care?
- Is there a diagnostic, management, systems, or communication lesson?

Patient Consent / Local Requirements

- Local process checked:
- Consent status:
- Supervisor responsible:
- De-identification checked:
- Journal/conference requirements checked:

Timeline

Time	Event
Day 0	Presentation
Day X	Investigation/intervention
Day X	Outcome/follow-up

Case Summary

Background

Briefly explain the clinical context and why this case is relevant.

Case Presentation

Include only necessary details:

- age range if appropriate
- relevant history
- presenting symptoms
- examination
- investigations
- diagnosis
- management
- outcome/follow-up

Discussion

Cover:

- what is already known
- what is unusual or important
- comparison to existing literature
- clinical learning point

- limitations

Learning Points

- Learning point 1
- Learning point 2
- Learning point 3

References

Use the target journal style.

Audit / QI Project Template

Project Title

[Process/standard] in [setting]: an audit / quality improvement project

Problem

What clinical or system problem are you addressing?

Standard

What guideline, policy, benchmark, or standard are you comparing practice against?

Examples:

- local hospital policy
- national guideline
- specialty society recommendation
- department target

Aim

By [date], improve/measure [process/outcome] in [population/setting] from [baseline if known] to [target if known].

Setting

- hospital:
- department:
- patient group:
- time period:

Governance

- project category: audit / QI / research / unsure
- local approval needed:
- supervisor:
- data access plan:
- patient consent considerations:

Measures

Process Measures

- measure 1:
- measure 2:

Outcome Measures

- measure 1:
- measure 2:

Balancing Measures

- possible unintended consequence:

Data Collection Plan

- inclusion criteria:
- exclusion criteria:
- data source:
- sample size:
- variables:
- who collects data:
- storage:

Intervention

If QI:

- intervention:
- who will implement:
- when:
- expected barrier:
- how success will be measured:

Output Plan

- department presentation:
- poster:
- conference:
- manuscript:
- CV evidence:

Next Actions

- action 1:
- action 2:
- action 3:

Abstract Template

Title

[Clear, specific title]

Background

1-2 sentences:

- What is the problem?
- Why does it matter?
- What gap does this project address?

Aim

This project aimed to [measure/evaluate/describe/compare/improve] [topic] in [population/setting].

Methods

Include:

- design
- setting
- population
- time period
- data source
- key variables
- analysis method
- approval/governance if relevant

Results

Include:

- sample size
- main numerical findings
- secondary findings
- important comparisons

Do not invent results if data is not complete.

Conclusion

Include:

- main takeaway
- implication
- next step

Before Submitting

Check:

- word count
- author list
- affiliations
- supervisor approval

- governance/ethics wording
- conference formatting
- conflict of interest
- references if required

Manuscript Section Template

Title Page

- title
- authors
- affiliations
- corresponding author
- word count
- abstract word count
- keywords

Abstract

Use the target journal format.

Introduction

Paragraph 1:

- establish clinical/research problem

Paragraph 2:

- summarise current evidence

Paragraph 3:

- identify gap and aim

Methods

Include:

- study design
- setting
- participants/sample
- inclusion/exclusion criteria
- data source
- variables
- outcomes
- statistical analysis
- ethics/governance

Results

Include:

- sample description
- primary findings
- secondary findings
- tables/figures

Discussion

Paragraph 1:

- main finding

Paragraph 2:

- comparison to literature

Paragraph 3:

- implications

Paragraph 4:

- limitations

Paragraph 5:

- conclusion/future work

Tables and Figures

- Table 1: baseline characteristics
- Table 2: main results
- Figure 1: flow diagram/process/results

References

Check target journal style.

Project	Type	Specialty	Supervisor	Institution	Status	Current Blocker	Next Action	Next Action Date	Target Output	Target Date	Approval Status	Your Role	Priority
Example antibiotic timing audit	Audit/QI	Surgery	Dr Example	Example Hospital	Idea	Need supervisor confirmation	Email consultant	2026-06-10	Department presentation	2026-08-01	Not started	Student contributor	High
Example unusual imaging case	Case report	Radiology	Dr Example	Example Hospital	Contacted	Awaiting reply	Follow up	2026-06-14	Case report abstract	2026-09-01	Consent/check local process	Literature search and draft	Medium
Example AI triage review	Literature review	Emergency	Dr Example	University	Scoping	Question too broad	Narrow PICO	2026-06-12	Conference poster	2026-10-01	N/A until design confirmed	Search and matrix	Medium

Name	Title	Specialty	Institution	Source	Relevant Work	Email	Date Contacted	Follow-Up Date	Response	Meeting Date	Possible Project	Status	N
Dr Example	Consultant	Surgery	Example Hospital	Placement	Audit/QI in perioperative care	example@email.com	2026-06-01	2026-06-10	No reply yet		Antibiotic timing audit	Contacted	R w yc on cc
A/Prof Example	Academic	Emergency	Example University	PubMed	AI triage papers	example@email.com	2026-06-02	2026-06-12	Interested		AI triage review	Warm lead	R pi be m

Citation	Year	Study Design	Population	Setting	Intervention_or_Exposure	Comparator	Outcome	Key Finding	Limitations	How It Helps This Project	Quality_Notes
Author et al. Example title	2024	Retrospective cohort	Adults with condition	Tertiary hospital	Exposure	Comparator	Primary outcome	Main finding	Small sample	Supports background	

Target	Type	Specialty	Location_or_URL	Submission_Deadline	Event_or_Publication_Date	Word_Count	Format	Fees	Eligibility	Status
Example State Research Day	Conference	General	URL	2026-07-01	2026-09-01	250	Abstract	Check	Students allowed	Considering
Example Case Report Journal	Journal	General	URL			1500	Case report	Check APC	Case reports accepted	Considering

Project	Person	Role	Contribution	Dates	Agreed_Authorship_Position	Confirmed_By	Evidence_Link	Notes
Example antibiotic timing audit	Your Name	Medical student	Literature search, data extraction, abstract draft	2026-06 to 2026-08	To be discussed	Dr Example		Update after supervisor discussion
Example antibiotic timing audit	Dr Example	Supervisor	Project design, governance, final review	2026-06 to 2026-08	Senior/corresponding author	Dr Example		

Study_ID	Patient_or_Record_ID_Deidentified	Inclusion_Check	Exclusion_Check	Age_Group	Sex	Date_Range	Variable_1	Variable_2	Variable_3
1	ID001	Yes	No						

Market Research and Offer Notes

This is a working research note for building the ResearchStack Starter product.

Observed Market Pattern

Medical research education offers tend to sell one of four outcomes:

1. start research without confusion
2. get published faster
3. learn academic writing
4. get mentorship/review/support

The strongest offers are not just "templates." They combine:

- step-by-step path
- practical templates
- checklists
- scripts
- examples
- confidence for beginners
- clear promise around output

Useful Competitor / Inspiration Notes

Your Research Coach / Dr Simona Ippoliti

Public positioning focuses on helping doctors, medical students, and healthcare professionals publish despite busy clinical work. The offer ladder includes lower-ticket research foundations and higher-ticket writing/publishing systems with tools, templates, AI support, and review/coaching.

What to learn:

- strong beginner pain: "I do not know where to start"
- clear pathway language
- practical templates matter
- writing and publication are premium outcomes
- coaching/review pushes the price higher

What not to copy:

- do not overpromise publication
- do not rely on generic "get published" language only
- keep Australian student context as our difference

Research Mentorship / Publication Programs

Many programs focus on systematic reviews, meta-analyses, case reports, or "zero to publication" mentorship.

What to learn:

- research beginners want someone to reduce ambiguity
- high-ticket offers usually include active mentorship or direct project participation
- self-serve products need to be cheaper unless the assets are extremely strong

Student Pain from Forums and Public Discussions

Repeated themes:

- students feel research is important but hard to enter
- some are wary of pay-to-publish or guru-style offers
- students want legitimate supervisors and real outputs
- authorship expectations are confusing
- projects often stall

- Australian students care about CV relevance for specialty training

Product implication:

- be honest and trust-building
- position as structure, not guaranteed publication
- teach students how to avoid vague/stalled projects
- include authorship and contribution tracking
- include "what to ask before joining" scripts

Differentiation for The MedStack

ResearchStack Starter should be:

- Australia-aware
- medical student specific
- research-to-CV focused
- practical before academic
- built around scripts, trackers, and decision support
- aligned with the MedStack OS ecosystem

Pricing View

Self-Serve Version

Likely launch range:

- AUD 197 for ResearchStack Starter
- AUD 249-299 if bundled with strong templates, sheets, and AI prompt vault

Bundle With MedStack OS

Possible offer:

- MedStack OS Complete Bundle: AUD 199 founding access
- Includes full OS plus Research Starter Pack, CV Evidence Pack, Scripts Vault, and PDF checklists

Higher-Ticket Future Version

AUD 499-999 becomes more realistic if it includes:

- recorded walkthroughs
- supervisor email review
- project idea review
- CV/research profile review
- office hours
- cohort support
- direct feedback on abstract/manuscript sections

Source Links To Revisit

- Your Research Coach: <https://yourresearchcoach.com/>
- Your Research Coach products: <https://yourresearchcoach.com/products>
- NHMRC National Statement: <https://www.nhmrc.gov.au/about-us/publications/national-statement-ethical-conduct-human-research-2023>
- NHMRC QA/evaluation ethics guidance: <https://www.nhmrc.gov.au/sites/default/files/documents/attachments/ethical-considerations-in-quality-assurance-and-evaluation-activities.pdf>
- EQUATOR reporting guidelines: <https://www.equator-network.org/reporting-guidelines/>
- RACS generic selection requirements: <https://www.surgeons.org/become-a-surgeon/set-selection-requirements-process-and-application/generic-eligibility-requirements-for-selection>
- General Surgeons Australia research requirement: <https://generalsurgeons.com.au/home/for-trainees/training-program/set-program/research-requirement/>

ResearchStack Starter Sales Page Draft

Hero

ResearchStack Starter

The practical research launch system for Australian medical students who want to find projects, contact supervisors, and build CV-worthy research outputs without guessing.

Button:

Get ResearchStack Starter

Secondary:

See what is inside

Pain Section

Medical students are told research matters.

But nobody shows you:

- who to email
- what to say
- what project type to start with
- what counts as a real output
- how to avoid stalled projects
- how to track your contribution
- how audit, QI, ethics, and publication fit together in Australia

So most students either wait too long, send vague emails, join unclear projects, or collect random tasks that never become a CV point.

Product Section

ResearchStack Starter gives you the structure to start properly.

Inside, you get:

- 30-day research sprint
- project type playbook
- supervisor outreach scripts
- follow-up and rescue scripts
- literature search starter
- audit/QI/case report templates
- abstract and manuscript templates
- research pipeline tracker
- supervisor CRM
- literature matrix
- authorship and contribution tracker
- conference/journal tracker
- AI prompt vault
- Australia-specific ethics/QI orientation

Outcome Section

By the end, you should have:

- a realistic project direction
- a shortlist of supervisors
- emails ready to send

- a live research pipeline
- one drafted project brief
- one next action for this week
- a way to turn research work into CV evidence

Trust Section

No publication promises.

No shortcuts around ethics or authorship.

No vague "just network more" advice.

Just the system most medical students wish someone gave them before they started emailing consultants.

Bundle Section

Best with The MedStack OS.

Use ResearchStack Starter to build the research pipeline.

Use The MedStack OS to track your CV evidence, applications, rotations, and career progress in one place.

FAQ

Is this only for Australian students?

It is built with Australian medical students in mind, especially around audit/QI, hospital governance, and specialty CV thinking. The general research process is still useful elsewhere, but students outside Australia should check their local ethics and training requirements.

Does this guarantee publication?

No. It helps you start properly, find opportunities, communicate professionally, and track your work. Publication depends on the project, supervisor, approvals, data, writing, journal/conference decisions, and follow-through.

Is there video content?

Not in the starter version. This is designed as a practical pack of guides, templates, trackers, scripts, and prompts.

Do I need prior research experience?

No. It is built for students who are starting from zero or who have tried to start but feel stuck.

Is this medical, legal, or ethics advice?

No. It is educational and organisational. Always check your supervisor, university, hospital research office, and HREC requirements.

ResearchStack Content Seeds

These can become Instagram reels, carousels, TikToks, or website sections.

Reel 1: Why Most Med Student Research Goes Nowhere

Hook:

Most medical students do not fail research because they are lazy. They fail because the project has no next action.

Points:

- vague supervisor
- vague question
- vague authorship
- vague output
- no follow-up date

CTA:

Build a research pipeline before you chase publications.

Reel 2: The Email That Gets Ignored

Hook:

If your research email says "I am interested in research opportunities," that is probably why nobody replies.

Better structure:

- who you are
- why them
- what topic
- what you can help with
- easy next step

CTA:

Use a specific email, not a desperate one.

Reel 3: Best First Research Projects

Hook:

If you are a med student starting research, do not begin with a huge prospective study.

Better first projects:

- case report
- audit/QI
- focused literature review
- medical education project

CTA:

Pick the project with the shortest path to a real output.

Reel 4: Research Red Flags

Hook:

Before joining a research project, ask these questions or you may waste six months.

Questions:

- What is the output?

- What is my role?
- What approval is needed?
- Who is supervising?
- What is the next action?
- How is authorship decided?

Reel 5: Audit vs Research

Hook:

Med students hear "just do an audit" all the time, but nobody explains what that actually means.

Points:

- audit compares practice to a standard
- QI improves a process
- research aims to generate broader knowledge
- local approval still matters

CTA:

Ask the governance question before touching data.

Reel 6: CV Evidence

Hook:

Research does not help your CV if you cannot prove what you actually did.

Track:

- project title
- supervisor
- your role
- output
- date
- evidence
- referee/contact

CTA:

Your future CV is built from today's evidence log.

Carousel 1: How To Start Research in Medical School

Slides:

1. Stop waiting for someone to hand you a project.
2. Pick 1-2 specialties.
3. Choose a realistic project type.
4. Build a 10-person supervisor list.
5. Read one thing they have worked on.
6. Send five specific emails.
7. Track every follow-up.
8. Ask about approvals and authorship early.
9. Aim for one output.
10. Use ResearchStack Starter.

Carousel 2: The Supervisor Meeting Checklist

Slides:

1. What project are we doing?
2. What is the research question?
3. What approval do we need?

4. Can I access data?
5. What is my role?
6. What is the output?
7. What is the timeline?
8. How will authorship be decided?
9. What is my first task?
10. When should I follow up?